SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma

Advanced melanoma · NCCP 00551 · version 7a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Schedule
Up to four combination infusions, 21 days apart, then nivolumab alone every two or four weeks as prescribed; four stamps record combination doses only.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Bowel, lung, liver and kidney inflammation
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Severe skin reactions
- ☐ Rare heart, muscle, nerve or eye inflammation
- ☐ Infusion or injection reactions

Ipilimumab
CTLA-4 CHECKPOINT IMMUNOTHERAPY

- ☐ Bowel inflammation and severe diarrhoea
- ☐ Liver and skin inflammation
- ☐ Thyroid, pituitary and adrenal problems
- ☐ Rare nerve, eye, muscle or heart inflammation
- ☐ Infusion reactions

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00551 v7a.

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Developed by Dr Paul O'Brien